



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ايزيس فيليب عزيز صموئيل

العنوان : ~

المحافظة : القاهرة

رقم البطاقة : 25704110101807 رقم التليفون : 01272508688 تاريخ تسجيل الإستمارة : 24-09-2026

العمر (تاريخ الميلاد) : 11-04-1957 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ايزيس فيليب عزيز صموئيل National ID: 25704110101807	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name: Isis Philip Aziz Samuel	Date : 15/06/2022
Exam : MAMMOGRAPHY BOTH	Age : 65 Year(s)
Medical ID: 2413/2018	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS of pathologically proved infiltrating duct carcinoma.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast: UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring.
- Complementary ultrasound also showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with no evidence of recurrent mass lesions.
- Left UOQ IMLN.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex
- Nonspecific bilateral axillary LN, with preserved fatty hila and uniform cortical thickness
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel with free operative bed with no evidence of recurrent mass lesions (BIRADS 2).
- Left breast benign finding (BIRADS 2).

Yours truly

Signature Dr. Nehal Hamdy

15/06/2022 5:34 PM

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ابريس
فيليب عزيز صموئيل
١٤٧ الحى ١١ مج ١
الشيخ زايد - الجيزة

١٩٥٧/٠٤/١١
٢٥٧.٤١١.٠١.١٨.٧
IV8064795

٢٠٢١/٠٤/١١ ٢٥٧.٤١١.٠١.١٨.٧

ربة منزل
أنثى
مسجلة
أرملة

البطاقة سارية حتى ٢٠٢٨/٠٤/٠٦

Name : Mrs Esseis feleip
Date: 17 March 2026

Neck ultrasound examination

Findings

Thyroid gland

- Diffuse thyroid gland coarse texture showing normal vascularity no right lobe nodules or masses seen, yet left lobe shows lateral superior small nodule 4x5mm showing central echogenic foci of calcification

Composition	Solid	1
Echotexture	Hyperechoic	2
Margin	Smooth-ill defined	0
Shape	Wider than taller	0
Extra thyroid component	-ve	0
Calcifications	2	

- No gross retrosternal extension
- Normal movement with swallowing
- Gland measurement
 - Left lobe measures about 3cc
 - Right lobe measures about 4.2 cc volume
 - Isthmus= 4mm
- Bilateral few small cervical lymph nodes, all are oval shaped with preserved hila likely non specific
- Other neck structures;
 - Trachea is centralized
 - Normal neck vasculature
 - Normal mobility of vocal cord on phonation

Opinion

Chronic thyroid disease

Left lobe thyroid small nodule TIRAD, III IV for follow up

Best regards
Dr. Abubakr Hejazi MSc FEBR

Patient Name: Eziez Feliep Aziez	Date : 17/03/2026
Exam : ultrasound ABDOMEN& PELVIS	

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

· **Liver: -**

- Average in size (15.5cm at Rt MCL) normal parenchymal echo pattern
- Portal vein is patent and not dilated. No IHBRD

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average in size, homogenous.

· **Kidneys: -**

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: masked by gases.

No ascites

Urinary Bladder: -

- Not distended enough however no stones or diverticular out pouching

Uterus and adnexal

no pelvic/ adnexal masses

Opinion:

- Much colonic gaseous distention.

Dr Mahmoud Ashour

Dr Abubakr Hejazi MSc FEBR



Patient Name: Isis Philip Aziz Samuel	Date : 15/06/2022
Exam : C.T CHEST	Age : 65 Year(s)
Medical ID: rt CBS WITH RECURRENT DYSYPNEA FOR M	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Focal skin irregularity/scarring at the upper aspect of the right breast.
- Bilateral pulmonary apical fine reticulations as well as tiny right apical calcific nodules and tiny calcified pre-vascular lymph node, likely post-granulomatous sequel.
- No pulmonary sizable nodules, masses or consolidations.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Right thyroid tiny hypodense nodule.
- Small sliding hiatus hernia.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***No CT evidence of pneumonia.***

Dr. Amr Farouk, M.D.

Patient Name : ايزيس فيليب عزيز صمونيل

Age : 50 Y

Referred By Prof. Dr. : موسى حلمي

Clinical Diagnosis : Right breast mass

Nature of Specimen : Tru cut biopsy

Receiving Date : 28-05-2018

Analysis ID : 294923

Gender : Female

Delivery Date : 29-05-2018

PATHOLOGY REPORT

Gross :

Multiple grayish yellow cores, longest measured 0.5 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed cores of tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation. The mitotic index is low. There is marked desmoplasia. There is lymphocytic infiltrate about 40 %.

No tumoural necrosis.

No intraductal component.

Diagnosis :

Right breast mass, Tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE II.

Elia
29/5/2018

1/1

Dr.

Elia Anis Ishak

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Dr.

Shady Elia Anis

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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايزيس فيليب عزيز صموئيل

Age : 62 Y

Referred By Prof. Dr. : هاني وليم زكي

Clinical Diagnosis : Breast mass

Nature of Specimen : Referred blocks

Receiving Date : 02/08/2018

Analysis ID : 310924

Gender : Female

Delivery Date : 05/08/2018

PATHOLOGY REPORT

Gross :

Three blocks labelled as [4321].

Microscopic :

Sections prepared from the blocks received revealed tumour tissue formed of irregular groups of malignant ductal cells, showing moderate nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation. The mitotic index is moderate. There is peritumoral lymphocytic infiltrate about 10%. There is marked desmoplasia. There is perineural invasion. There are scattered tumour emboli. There is focal tumoral necrosis. Sections from lymph nodes revealed metastatic deposits in 8 out of 15 lymph nodes. No intraductal component.

Diagnosis :

Referred blocks, INFILTRATING DUCT CARCINOMA, GRADE II, NO INTRADUCTAL COMPONENT, POSITIVE AXILLARY LYMPH NODE METASTASIS [L.N. 8/15] (pN2a).

05/08/2018

Dr.

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Patient Name : ايزيس فيليب عزيز صمونيل

Age : 62 Y

Referred By Prof. Dr. : هانى وليم زكى

Clinical Diagnosis : Breast mass

Nature of Specimen : Referred blocks

Receiving Date : 02/08/2018

Analysis ID :  310925

Gender : Female

Delivery Date : 05/08/2018

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP, DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- Estrogen receptors
- Progesterone receptors.
- HER2.

Results :

Sections treated for Estrogen receptors revealed NEGATIVE Nuclear staining by chromogen.

Sections treated for Progesterone receptors revealed NEGATIVE Nuclear staining by chromogen.

Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANOUS STAINING (CHICKEN WIRE) IN MORE THAN TEN PERCENT OF TUMOR CELLS.

Conclusion :

Referred blocks, TUMOUR CELLS ARE:

NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8),

NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8),

STRONGLY POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+


05/08/2018

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Age : 62 Y

Referred By Prof. Dr. : هاني وليم زكي

Clinical Diagnosis : Breast mass

Nature of Specimen : Referred blocks

Receiving Date : 02/08/2018

Analysis ID :  310924

Gender : Female

Delivery Date : 05/08/2018

PATHOLOGY REPORT

Gross :

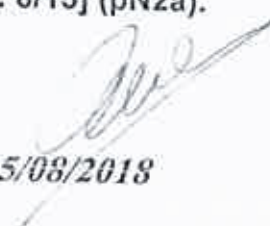
Three blocks labelled as [4321].

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05/08/2018

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Patient Name : ايزيس فيليب عزيز صمونيل

Age : 62 Y

Referred By Prof. Dr. : هانى وليم زكى

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NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8),

STRONGLY POSITIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 3+


05/08/2018

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٦